

WHY PEOPLE CLICK

A Behavior-Based Cybersecurity Awareness Campaign – Elite Healthcare

A Cyber Psychology Project

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Program Overview

Most security awareness programs are built to deliver information: what fishing looks like, what a suspicious link looks like, and what to report. Elite health care employees who click phishing links are rarely informed that annual compliance training already covers the basics. This program is built on a different premise: Clicking is a behavior, not a knowledge gap, and behavior change requires more than a side deck once a year.

The program below is designed around a behavior-based model rather than a content delivery model. It combines a short facilitator-led training session, a one-page visual reference, a structured facilitator guide, and pre- and post-training questionnaires to measure actual knowledge gained, with a four-week rollout that includes specific, measurable indicators of success.

Design principle: every component of this program is built to change what an employee does in the 10 seconds before a click, not just what they can recite in a quiz, because those two things are miserably different.

2. Why People Click: The Behavioral Case for the Program

Healthcare is the single most-phished industry sector. Industry benchmarking data puts the healthcare and pharmaceutical sectors' baseline phishing-prom rate at 41.9%, the highest of any industry measured, meaning roughly two and five untrained healthcare employees will interact with a simulated phishing attempt before any training occurs. Separately, the median time between a phishing email landing in an inbox and a user clicking it has been measured at 21 seconds. Not enough time for a person to consciously apply a training module; there is only enough time for a fast, automatic judgment.

2.1 The Triggers Attackers Rely On

Trigger	How it appears in a phishing email
Authority	Cinder appears to be a CMO, IT director, or compliance officer. Employees are trained from day one to comply with authority, not interrogate it.
Urgency	“Your account will be locked in 2 hours.” Urgency compresses decision time and pushes the response into system 1.
Fear	“Unusual login detected” or “Patient record access flagged” triggers a threat response that prioritizes fast resolution over verification.
Curiosity	“Updated PTO policy attached” or “ You've been mentioned in a report” exploits the drive to resolve an information gap.
Habit and banner blindness	Repeated exposure to legitimate systems notices trains employees to click through routine-looking prompts without reading them.
Optimism bias	“ It won't be me,” the world documented tendency to rate personal risk lower than the statistical average, even after training.

3. Program Design: A Behavior-Based Framework

The program is built on the COM-B model of behavior change (Capability, Opportunity, Motivation, Behavior), which holds that behavior changes only when all conditions are addressed together. A knowledge-only training program addresses capability alone and predictably produces only a short-lived improvement.

4. Training Content Outlined

Module 1: Open the real numbers

Open with a healthcare sector 41.9% baseline click rate and a 21-second median click time. The goal is not to scare the room, but to reframe the topic. This is not about knowledge tests; it's about a 10-second window that happens to everyone.

Module 2: The Six Triggers

Walk through the trigger table and section 2.1, using two or three real (redacted) examples of fishing emails that employees at Elite Healthcare have actually received after rooms, to identify which triggers each example uses before revealing the answer.

Module 3: The 3-Second Hover-and Check

Teach one specific repeatable physical action before clicking any link, hover to reveal the true URL, check the sender domain matches exactly, and check for one of the six triggers above. This is the single behavior the entire program is designed to install.

Module 4: Reporting Without Shame

Explicitly state that reporting a suspicious email, even if the employee has already clicked, is treated as a positive action, not a disciplinary event. Walk through the exact reporting step (button, forwarding address, or ticket process) to avoid ambiguity.

Module 5: Close and Committed

In with the post-training questionnaires section 7, and a show of hands committed to the Hoover and click habit, and distribute a one-page infographic section 5 for employees to keep at their workstation.



6. Facilitator Guide & Delivery Model

This program is designed to be delivered by a department lead, IT security staff member, or HR trainer with no specialized security background beyond this guide.

6.1 Session Structure

Time	Segment	Facilitator Notes
0:00-0:05	Open The Real Numbers	Lead with the 41.9% and 21-second statistics before anything else. Do not open with rules or policy; open behavioral reality.
0:05-0:15	The Six Triggers	Use real radioactive examples from Elite Healthcare's own inbox if available; generic examples are weaker than internal ones.
0:15-0:25	The Three Second Hover and check	Physically demonstrate hovering over a link on a shared screen. Have two or three participants practice out loud.
0:25-0:30	Reporting without Shame	Stay especially in these words or similar "Reporting is never a disciplinary event, even if you are ready clicked." Repetition of this line matters more than any other single sentence in the session.
0:30-0:35	Close and Post-Questionnaire	Distribute the post-training questionnaire and the one-page reference sheet before participants leave the room.

6.2 Delivery Tips

- Keep sessions to 15-20 participants; smaller groups produce more honest questions about real emails employees have already clicked.
- Never use a real employee's name when discussing a pass click incident; even anonymized details can be identified in a small department.
- Schedule sessions department-by-department rather than facility-wide, so examples and language can be tailored (billing phishing differs from clinical phishing).
- Repeat the "no penalty for reporting" line at both the start and the end of the session. Motivational research shows that a single mention is often not retained under stress.

7. Pre-and Post-Training Knowledge Questionnaires

The same 10 items are used before and after training in a reordered sequence so that knowledge gain can be measured item by item rather than only as an aggregate score. Each Item is scored correct/incorrect. The Delta between pre- and post-scores is the primary indicator of knowledge gain (section 8).

7.1 Pre-Training Questionnaire (administered before Module 1

#	ITEM
1	True/False: If an email looks like it's from my supervisor, I don't need to check the sender's actual email address
2	What is the fastest way to check where a link actually leads before clicking it?
3	True/False: I will be disciplined if I report an email I already clicked on.
4	Name two emotional triggers that phishing emails commonly use to rush a decision
5	True/False: A sense of urgency in an email ("act within 2 hours.") is a normal, low-risk business practice.
6	Where do you report a suspicious email at Elite Healthcare?
7	True/false: Only emails with obvious spelling errors are phishing attempts
8	What should you do if an email asks you to verify account access by clicking a link?
9	True/False: A matching company logo in an email confirms it is legitimate.
10	On average, how long does it take most people to click a phishing link after opening the email?

7.2 Post-Training Questionnaire (administered after Module 5: reordered)

#	ITEM
1	On average, how long does it take most people to cook a phishing link at the opening of the email? (matches Pre #10)
2	True/False: A sense of urgency in an email (“act within 2 hours”) is a normal, low-risk business practice (matches pre #5)
3	What is the fastest way to check a link actually leaves before clicking it (matches Pre # 2)
4	True/False: A matching company logo in an email confirms it is legitimate (matches Pre #9)
5	Name two emotional triggers that phishing emails commonly use to rush a decision. (matches Pre #4)
6	True/False: I will be disciplined if an email I already clicked on (matches Pre #3)
7	What should I do if an email asks you to verify account access by clicking a link (matches Pre #8)
8	True/False: Only Emails with obvious spelling errors are phishing attempts (matches Pre #7)
9	Where do you report a suspicious email at Elite Healthcare? (matches Pre #6)
10	True/False: If an email appears to be from my supervisor, I don't need to verify the sender's actual email address. (matches Pre #1)

Answer Key basis: Items 1,3,5,7, and 9 (True/False) are all scored “False”; each describes a common false assumption that the training is specifically designed to correct. Open-response items (2, 4,6, 8, 10) are scored against the Hoover and check method, the authority/urgency/fears/curiosity triggers set, the designated reporting channel, and the 21-second statistic, respectively.

8. Four-Week Roll-Out Plan Measurable Indicators

The rollout is staged over four weeks to allow a true baseline measurement, a training intervention, and two follow-up measurement points, rather than a single training event with no way to confirm that behavior actually changed.

Week	Activity	Audience/Scope	Measurement Indicator
Week 1	Baseline phishing simulation (unannounced); pre-training questionnaire distributed to a sample group	All departments, staggered by shift	Baseline click-through rate (%); baseline questionnaire score (avg./10)
Week 2	Facilitator-led training sessions were delivered department-by-department, and a post-training questionnaire was administered the same day	All employees, 15-20 per session	Session attendance rate (%); pre/post questionnaire score delta
Week 3	First follow-up phishing simulator using a different trigger type than Week 1, one-page reference posted at work stations	.All departments	Week 3 click-through rate vs. Week 1 baseline (% change); reporting rate (% of recipients who reported instead of clicking)
Week 4	Second follow-up phishing simulation; manager debrief and department-level scorecards distributed	All departments	Week 4 click-through rate vs. Week 1 baseline; reporting rate trend across Weeks 1,3, and 4

8.1 Success Indicators Summary

Indicators	Baseline (Week 1)	Target (Week 4)
Simulated phishing click-through rate	Health care industry average; 41.9%	Reduced by at least 50% relative to the internal Week 1 baseline
Knowledge questionnaire score (avg./10)	Measured Week 1	Statistically significant increase, measured same-day post-training
Suspicious email reporting rate	Measured Week 1	N/A
N/A	N/A	Increase across Weeks 1,3,4, independent of click-rate change
Repeat-clicker rate (employees who click in more than one simulation)	Measured across Weeks 1,3,4	Identify targeted, non-punitive one-on-one coaching rather than disciplinary action.

9. Conclusion

A single annual training module changes what employees can recite, not what they do, and the 21 seconds after a fishing email lands in their inbox, this program is built around that distinction; It measures knowledge gain and click rate change separately, because they do not move together automatically. The four-week structure exists specifically to confirm that a behavior Change actually occurred, not just that a training session was attended.

Fictional program developed for Elite Healthcare as part of a Cyber psychology portfolio project